

Specialty Clinic Pharmacist Collaborative Model

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Background

- Yale New Haven Health System (YNHHS) has a growing ambulatory care pharmacy presence
- One area of opportunity is expansion of fully integrated ambulatory care pharmacists into specialty disease state clinics and closer partnership with the health system pharmacy
- Patients in these clinics are often prescribed specialty medications and require a higher level of coordination for dispensing and patient education
- A similar model showed that integration of a pharmacist decreased clinician burden, decreased time to medication initiation, and improved continuity of care for patients¹

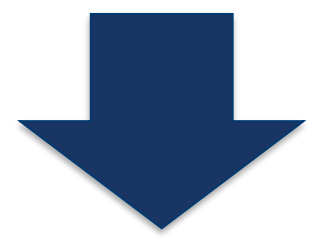
Objectives

- Expand health system specialty pharmacy prescription clinical continuity (determined by the number of prescriptions sent to health system pharmacy) and support by implementing a decentralized specialty practice integrated model for clinical services in target sites.

Methods

Key Service Implementation Steps

Development of innovative collaborative practice agreement (CPA)	Updates to existing credentialing policy
Collaboration with stakeholders	Compliance with regulatory standards
New workflow	Marketing strategies
Enhancements/optimizations to electronic health record (EHR)	



Service Evaluation:

Primary Endpoints: clinical continuity and number of signed referrals
Secondary Endpoints: disease state clinical outcomes

- Rheumatology: RAPID 3 improvement
- Digestive Health: disease flare reduction
- Hepatitis C: sustained viral suppression

Service Implementation Timeline

Aug – Sept 2020	- Environmental analysis - Stakeholder identification - Project planning
Oct 2020 – Jan 2021	- Stakeholder engagement - Development of new CPA - Updates to credentialing policy - Recruitment - EHR optimizations
Dec 2020 – April 2021	- Finalized CPA - Continued updates to credentialing - Creation of standardized workflow - Updates to documentation in EHR
March – May 2021	- Approval of new CPA - CPA Live in target clinics - Finalized workflow - Marketing strategies

CPA Development

CPA Sections: disease state, therapeutic class of medications, ability to initiate/ modify/ refill/ discontinue therapy, laboratory monitoring, guideline references



Scan QR code for sample CPA appendix

Marketing Strategies

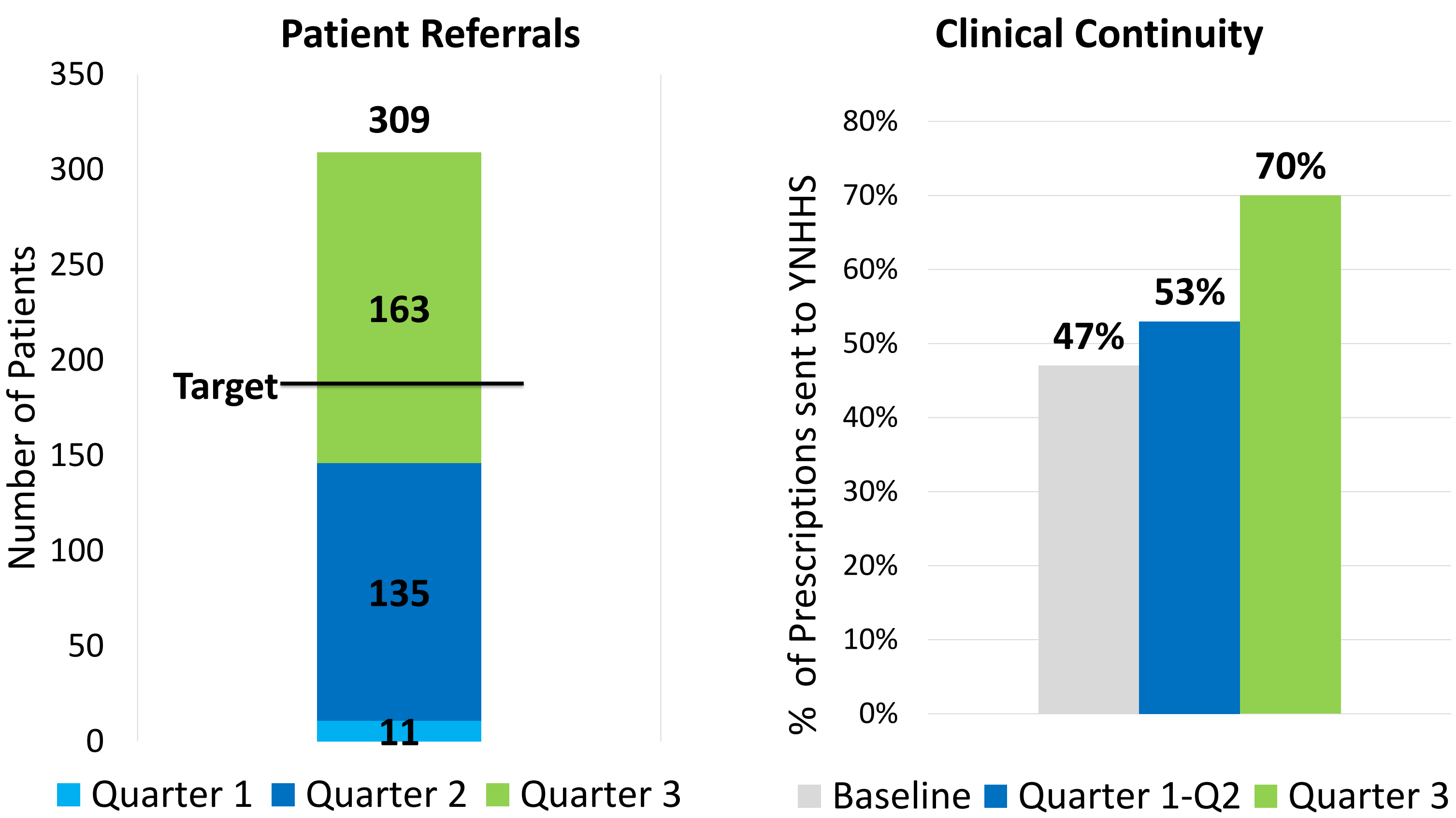
Patient brochure	Clinician brochure	Pharmacist introduction flyer
Website	Scripting	Stakeholder engagement

Marketed Pharmacy Services

Medication counseling	Disease state education and follow up
Administration teaching	Comprehensive medication management
Coordination of prescriptions with health system pharmacy	Lab monitoring

Results

Primary Endpoints



Overall Outcomes (In development)

Medication adherence	Lab test adherence
Immunization rates	Patient / clinician satisfaction
Clinical interventions	

Clinician Testimonials

“The addition of the clinical pharmacist has been **the best patient care improvement** in my 30 years in practice.”

“Pharmacists’ participation in our patient care has brought us to a **new level of care**. Experience navigating the complicated system of site of care and facility administration lends a new and important degree of **comfort that our patients are being well cared for.**”

“ Having a **pharmacist is crucial** in obtaining medication approval for these patients, negotiating insurance regulations, sites of service, and the proper medical monitoring required.”

“Relationship has **eased the burden** on our office staff, enhanced the difficult ordering process on Epic, and offered **tremendous clinical benefit** to our patients.”

Discussion

- Limited data is available in the pharmacy community about the implementation of pharmacists into specialty clinics
- This model has been proven to be successful and can be adapted to other health systems with specialty pharmacies
- The name of the dedicated ambulatory pharmacy clinic chosen was Medication Management Clinic (MMC) to tie in with national recommendations for reference to pharmacy services and to avoid confusion with the dispensing pharmacy
- The results of this model align with other reports showing that ambulatory care pharmacists can help decrease clinician burden and improve patient outcomes

Conclusions

- Expansion of integrated decentralized pharmacists into target practices has increased clinical continuity by 23% and number of pharmacist referrals exceeded target at 165%
- Clinicians have received pharmacists as vital members of the team

Barriers / Limitations

- Clinicians’ full understanding of an ambulatory care pharmacist’s role
- Existing CPA regulations required monthly documentation
- Model only developed in limited number of adult ambulatory care clinics
- Limited data due to time since implementation

Future Directions

- Continued expansion into additional specialty disease state clinics and review of endpoints
- Integration of advanced pharmacy technician roles
- Continued optimization of workflow

References: 1. Autumn Bagwell, Tara Kelley, Alicia Carver, Jennifer B. Lee, and Brandon Newman
Journal of Managed Care & Specialty Pharmacy 2017 23:8, 815-820.

Disclosure: The authors of this presentation have the following to disclose concerning possible financial or personal relationships with commercial entities that may have a direct or indirect interest in the subject matter of this presentation: Natasha Stroedecke PharmD, nothing to disclose; Jenna Lee PharmD, BCPS, BCACP, nothing to disclose; Martha Stutsky PharmD, BCPS, nothing to disclose; Kimberly Boothe PharmD, MHA, nothing to disclose; Kimhouy Tong PharmD, BCPS, nothing to disclose; Steph Luon PharmD, BCPS, BCACP, nothing to disclose; Esther Eom PharmD, nothing to disclose; Vinay Sawant RPh, MHA, MBA, nothing to disclose; Marie Renauer PharmD, MBA, BCACP, nothing to disclose
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